

NOURISH — Postpartum

Phase 3 of 6 — Rebuilding

Postpartum Week 25

"More potassium-rich foods, less excess sodium — unless your care team has given you different limits."

Weekly nourishment theme

Potassium, working alongside sodium for steady blood pressure

"More potassium-rich foods, less excess sodium — unless your care team has given you different limits."

Why it matters now

Potassium is an electrolyte your body uses to help regulate fluid balance, nerve signals, and muscle contraction — including your heart's own rhythm. It also plays a genuine, well-evidenced role in blood pressure: diets richer in potassium and lower in sodium are linked to a real reduction in blood pressure and stroke risk.

Your daily reference: ICMR-NIN gives a general adult potassium reference of 3,500 mg a day and a sodium limit/reference of about 2,000 mg a day, equivalent to roughly 5 g of salt from all sources — not 5 g of salt added on top of what's already in food. It also considers their relationship using a desirable sodium-to-potassium ratio expressed in molar units, not equal milligrams. The practical message is simpler than the maths: include potassium-rich whole foods regularly while avoiding excessive sodium from added salt, packaged foods, and restaurant meals. No separate pregnancy or lactation figure is published for potassium — the same reference applies at every stage. For women aged 19 years and older, NIH's US Adequate Intake is 2,600 mg a day when non-pregnant, 2,900 mg a day during pregnancy, and 2,800 mg a day while breastfeeding — slightly lower than pregnancy, a genuine feature of the NIH system, not an error. If you are under 19, use the age-specific guidance given by your maternity team. These are two different reference systems, presented honestly rather than reconciled into one number.

If you are breastfeeding or expressing milk: If you're breastfeeding, NIH's reference is 2,800 mg a day — slightly below the pregnancy figure. ICMR-NIN doesn't publish a separate breastfeeding figure, so its general 3,500 mg a day reference applies. A varied day containing legumes, vegetables, fruit, and dairy or other protein foods can contribute substantially without requiring exact counting.

If you are not breastfeeding: If you're not breastfeeding, potassium remains useful for everyday fluid balance and heart rhythm, and your daily reference returns to the general adult figure.

Where potassium comes from: Potassium is genuinely abundant across ordinary staples — legumes and dal, potato, curd, spinach, tomato, and a wide range of fruit (orange, mosambi, guava, papaya, banana, and others) all contribute meaningfully. Even in places where this has been studied directly, most people eat somewhat less potassium than the reference suggests, so leaning toward a variety of these foods regularly is genuinely useful, not just a formality.

A gentle note for this week: No upper limit has been set for potassium from food in either reference system, for someone with healthy kidneys — the body can usually regulate potassium from ordinary foods when kidney function is normal. The real caution is for anyone with reduced kidney function, or taking certain medicines (see the condition note below), where even ordinary potassium intake can build up. Potassium supplements can cause harm in the wrong dose or medical context — don't add one without your clinician's guidance.

This week's rhythm: Because potassium is spread naturally across so many ordinary foods, there's no need to plan around it specifically — a normal, varied pattern with legumes, vegetables, fruit, and dairy or other protein foods contributes substantially, without needing exact counting.

Each day below offers two options for every meal — pick whichever suits you. Potassium shows up naturally across most of this week's choices, so no single meal needs to carry the whole day.

If you are managing a health condition

- Kidney disease, or taking a medicine that can raise potassium: If you have chronic kidney disease, receive dialysis, have previously had high blood potassium, or take medicine that can raise potassium, follow your own laboratory monitoring and food guidance rather than this general chart. High potassium can sometimes cause no noticeable symptoms, so prescribed blood tests matter.
- High blood pressure or previous pre-eclampsia: Potassium-rich whole foods and avoiding excess sodium can support a blood-pressure-friendly eating pattern, but they don't treat postpartum hypertension or replace medicine and monitoring. Seek urgent assessment for severe or persistent headache, visual changes, chest pain, breathlessness, severe upper-abdominal pain, sudden marked swelling, or a blood-pressure reading at or above the urgent threshold your maternity team gave you.
- Thyroid conditions, diabetes, recent gestational diabetes, or PCOS: These don't require a special restriction on ordinary potassium-containing foods. Continue the eating pattern, medication, and monitoring already advised for your condition. Choose whole fruit more often than juice, and keep potato, fruit, rice, and roti portions consistent with the guidance already given to you.

Day 1 — Anchor

Potassium is an electrolyte your body uses to help regulate fluid balance, nerve signals, and muscle contraction — including your heart's own rhythm. It also plays a genuine, well-evidenced role in blood pressure: diets richer in potassium and lower in sodium are linked to a real reduction in blood pressure and stroke risk.

Breakfast — choose one:

- A. Vegetable poha with a whole orange, mosambi, guava, or another seasonal whole fruit.
- B. Boiled egg (cooked through) with whole-wheat toast and a banana.

Lunch — choose one:

- A. Whole-wheat roti with a rajma curry and a spinach sabzi.
- B. Chicken curry (cooked through) with rice and a potato sabzi.

Nourishment break — choose one:

- A. Roasted peanuts with a guava.
- B. A small bowl of curd with a seasonal fruit.

Dinner — choose one:

- A. Sweet potato and vegetable sabzi with a whole-wheat roti.
- B. A curry made with a locally available fish from the lower-mercury category, with rice and a tomato salad.

If you're still hungry: Whatever's easiest to reach for right now.

Day 2 — A gentle shift after pregnancy

NIH's reference for potassium actually eases slightly after pregnancy — 2,900 mg a day in pregnancy, 2,800 mg a day while breastfeeding. ICMR-NIN's general 3,500 mg a day reference doesn't change by stage. A varied day containing legumes, vegetables, fruit, and dairy or other protein foods can contribute substantially without requiring exact counting.

Breakfast — choose one:

- A. Ragi dosa with a small bowl of curd.
- B. Masala omelette (two eggs, cooked through) with whole-wheat toast.

Lunch — choose one:

- A. Bajra roti with a moong dal and a beans sabzi.
- B. Mutton curry (cooked through) with rice and a tomato-and-cucumber salad.

Nourishment break — choose one:

- A. An orange with roasted almonds.
- B. A small bowl of curd with sunflower seeds.

Dinner — choose one:

- A. Vegetable khichdi with a side of curd.
- B. Chicken curry (cooked through) with rice and a cabbage sabzi.

If you're still hungry: A familiar snack.

About your prenatal or postnatal supplement: Potassium supplements can cause harm in the wrong dose or medical context. Check prenatal, postnatal, electrolyte, effervescent, sports, and recovery products for potassium, and don't add a potassium-only product without clinical advice.

Day 3 — Where potassium naturally shows up

Potassium appears across legumes, vegetables, dairy or tolerated alternatives, and many whole fruits. Today is about rotating these ordinary foods rather than relying on bananas or counting every milligram.

Breakfast — choose one:

- A. Vegetable daliya with a banana.
- B. Egg bhurji with whole-wheat toast.

Lunch — choose one:

- A. Whole-wheat roti with a chana sabzi and a small bowl of curd.
- B. A curry made with a locally available fish from the lower-mercury category, with rice and a cucumber salad.

Nourishment break — choose one:

- A. Roasted cashews with a papaya.
- B. A small bowl of yogurt with a seasonal fruit.

Dinner — choose one:

- A. Whole-wheat roti, a masoor dal, and a mushroom sabzi.
- B. Mutton curry (cooked through) with rice and a cabbage sabzi.

If you're still hungry: A familiar snack.

If you have a diagnosed allergy: If you have a diagnosed egg, dairy, peanut, seed, or seafood allergy, avoid that food completely and choose one of the many other sources in this week's chart. If a food causes digestive discomfort rather than an allergy, use only forms and portions you already know you tolerate. Don't trial a problem food to pursue this week's nutrient.

Day 4 — Why more isn't the goal

No upper limit exists for potassium from food in a healthy adult — but medicines and salt substitutes change the picture.

Breakfast — choose one:

- A. Oats porridge with a mosambi.
- B. Chicken sandwich (whole-wheat bread, fully cooked chicken) with a side of tomato.

Lunch — choose one:

- A. Jowar roti with a rajma curry and a beans sabzi.
- B. Mutton curry (cooked through) with rice and a green-peas sabzi.

Nourishment break — choose one:

- A. Sunflower seeds with an orange.
- B. A small bowl of curd with almonds.

Dinner — choose one:

- A. Whole-wheat roti, a moong dal, and a potato sabzi.
- B. A curry made with a locally available fish from the lower-mercury category, with rice and a carrot salad.

If you're still hungry: A familiar snack.

Why more isn't the goal: Some medicines — including ACE inhibitors, ARBs, and potassium-sparing diuretics — can raise blood potassium, especially when kidney function is reduced. This isn't a complete medication list. Don't stop prescribed medicine or independently restrict nutritious foods. Ask your clinician or pharmacist before adding potassium supplements, electrolyte powders, or potassium-based salt substitutes.

About potassium-based salt substitutes: Some low-sodium, 'lite,' or mineral salts replace part of the sodium with potassium chloride. Don't switch to a potassium-based salt substitute solely for this week's theme. Check with your clinician or pharmacist first if you have kidney or heart disease, diabetes with kidney involvement, or take medicine that may raise potassium.

Day 5 — A naturally reassuring nutrient

Low potassium is rarely caused by one imperfect food day. It's more often associated with prolonged vomiting or diarrhea, certain medicines, or laxative overuse. Symptoms such as fatigue, constipation, or muscle weakness are nonspecific, so don't diagnose or treat yourself with potassium supplements. Follow your clinician's rehydration advice, and seek prompt care for persistent vomiting or diarrhea, inability to keep fluids down, markedly reduced urination, palpitations, fainting, confusion, chest symptoms, or severe or worsening weakness.

Breakfast — choose one:

- A. Vegetable uttapam with a small bowl of curd.
- B. Egg paratha (whole-wheat) with a side of curd.

Lunch — choose one:

- A. Whole-wheat roti with a masoor dal and a potato sabzi.
- B. Chicken curry (cooked through) with rice and a mushroom sabzi.

Nourishment break — choose one:

- A. Roasted chana with a banana.
- B. A small bowl of yogurt with sunflower seeds.

Dinner — choose one:

- A. Whole-wheat roti, a moong dal, and a cauliflower sabzi.
- B. Mutton curry (cooked through) with rice and a tomato salad.

If you're still hungry: A familiar snack.

Day 6 — Away from home

Roasted cashews, sunflower seeds, and roasted chana are naturally easy to carry today — no cold storage needed for these.

Breakfast — choose one:

- A. A whole-wheat vegetable sandwich with a slice of cheese.
- B. A pre-made boiled-egg sandwich (whole-wheat bread), carried in an insulated bag with an ice pack, kept cold until eaten.

Lunch — choose one:

- A. A roasted-chana-and-vegetable whole-wheat wrap, carried in an insulated bag with an ice pack, kept cold until eaten.
- B. A pre-made chicken sandwich (whole-wheat bread, fully cooked chicken), carried in an insulated bag with an ice pack, kept cold until eaten, with a tomato side.

Nourishment break — choose one:

- A. Roasted cashews, kept portable — no cold storage needed.
- B. A small flask of buttermilk, kept cold.

Dinner — choose one:

- A. A reheated moong dal-and-vegetable curry with rice.
- B. A reheated mutton curry with rice and a vegetable side.

If you're still hungry: Whatever's already within reach.

Food safety: Keep the egg, chicken, and cheese sandwiches, and the flask of buttermilk, refrigerated or in an insulated bag with an ice pack, and make sure they stay cold until you actually eat or drink them — discard anything that's been sitting out at room temperature for more than about two hours (about one hour in hot weather). Roasted cashews, sunflower seeds, and roasted chana are shelf-stable and don't need the same cold-chain care. Reheat the dal curry or mutton curry until steaming hot before eating.

Day 7 — Reflection

A quiet week to notice what already felt manageable, and what you'd want to carry into the weeks ahead.

Breakfast: Whichever breakfast felt best this week — worth repeating.

Lunch: Whichever lunch from this week you'd want again.

Nourishment break: Whichever snack worked best for you this week.

Dinner: Whichever dinner from this week you'd want again.

If you're still hungry: A familiar snack, whatever suits you.

Still in Rebuilding. A steady, varied pattern of meals does more for you here than any single perfect meal.

Gentle weekly reflection

Did learning about the sodium-potassium pairing change how you think about the salt and fruit already in your day?

Looking ahead

Next week, we'll look at another nutrient as you continue rebuilding your own strength.

Food safety basics

- Cook eggs, meat, and fish all the way through — no runny yolks or pink centres.
- Cook sprouts before eating them. Raw sprouts carry a real risk of bacteria that washing doesn't remove — skip them raw entirely.
- Choose pasteurized milk, curd, and paneer.
- Wash fruits and vegetables well.
- Use reliably safe drinking water. If the source is uncertain, boil it or use a purifier suitable for the local water risk, and avoid ice from unknown sources when eating out.
- Keep total caffeine under 200 mg a day if you're breastfeeding, unless your maternity team advises otherwise.
- Not drinking alcohol is the safest option while breastfeeding. If you choose to drink, allow at least two hours per standard drink before breastfeeding, and make sure a sober adult is available to care for the baby.
- Refrigerate cooked food promptly, and reheat leftovers until steaming hot, not just warm.
- Choose lower-mercury fish and cook it fully — two to three servings a week from varied types is a commonly used guideline, following any local advisories.